



Telangana Diagnostics

Department of Health, Medical & Family Welfare
Government of Telangana

Patient Name:	G ANITHA	Ref. Doctor:	Dr. Vikarabad CHC
Patient Id:	0002596036	Lab Id:	02260070355
OP Id:	HY-VIK-0226-00421	Sample Collection Date & Time:	11-02-2026 08:54 AM
Age/Gender:	50 Years/Female	Reporting Date & time:	11-02-2026 07:16 PM

CLINICAL BIOCHEMISTRY

Investigation	Result	Biological Reference Interval
Specimen: Serum		
Vitamin - B12 (Method: CLIA)	317 pg/mL	120 - 914 pg/mL
Interpretation Results may differ between laboratories due to variation in population and test method. Vitamin B12 is implicated in the formation of myelin, and along with Folate is required for DNA synthesis. The most prominent source of B12 for humans is meat while untreated fresh water can also be a source. Megaloblastic anaemia has been found to be due to B12 deficiency, a major cause being Pernicious anaemia due to poor B12 uptake resulting in below normal serum levels. Other conditions related to low B12 levels include iron deficiency anaemia, pregnancy, vegetarianism, partial gastrectomy, ileal damage, oral contraceptives, parasitic infestations, pancreatic deficiency, treated epilepsy and advancing age. The correlation of serum B12 levels and Megaloblastic anemia however is not always clear - some patients with high MCV may have normal B12 levels, while some individuals with B12 deficiency may not have megaloblastic anemia. Disorders renal failure, liver diseases and myeloproliferative diseases may have elevated vitamin B12 levels. For diagnostic purposes, the B12 results should be used in conjunction with other data : eg - symptoms results of other testing, clinical impressions etc. If the B12 level is inconsistent with clinical evidence, additional testing is suggested to confirm the result		

*** END OF REPORT ***

Please correlate clinically

Test Performed By : VVKB

Consultant Biochemist

Lab Address:



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CLINICAL BIOCHEMISTRY

RENAL FUNCTION TEST

Investigation	Result	Biological Reference Interval
Specimen:Serum		
Blood Urea (Method: Urease)	23.5 mg/dL	17 - 43 mg/dL
S.Creatinine (Method: Enzymatic)	0.70 mg/dL	0.55 - 1.02 mg/dL
Interpretation		

*** END OF REPORT ***

Please correlate clinically

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CLINICAL BIOCHEMISTRY

Investigation	Result	Biological Reference Interval
Specimen: Serum		
Vitamin - D3 (Method: CLIA)	35.2 nmol/l	Deficient < 50 Insufficient 50 - 75 Sufficient 75 - 250 Potential Toxicity > 250

Interpretation 1. Vitamin D is a steroid hormone involved in the intestinal absorption of calcium and the regulation of calcium homeostasis. Vitamin D is essential for the formation and maintenance of strong, healthy bones. 2. Vitamin D deficiency can result from inadequate exposure to the sun, inadequate alimentary intake, decreased absorption, abnormal metabolism, or vitamin D resistance. Recently, many chronic diseases such as cancer, high blood pressure, osteoporosis, and several autoimmune diseases have been linked to vitamin D deficiency.

*** END OF REPORT ***

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Patient Name: G ANITHA
Patient Id: 0002596036
OP Id: HY-VIK-0226-00421
Age/Gender: 50 Years/Female

Ref. Doctor: Dr. Vikarabad CHC
Lab Id: 02260070355
Sample Collection Date & Time: 11-02-2026 08:54 AM
Reporting Date & time: 11-02-2026 07:22 PM

CLINICAL BIOCHEMISTRY PROFILE - THYROID

Investigation	Result	Biological Reference Interval
Specimen:Serum		
Test T3 (Method: CLIA)	1.79 ng/mL	0.69-2.11 ng/mL
Test T4 (Method: CLIA)	9.81 ug/dL	5.4-13.35 ug/dL
Test TSH (Method: CLIA)	1.968 uIU/mL	0.465-4.68 uIU/mL
*** END OF REPORT ***		

Please correlate clinically

Test Performed By : VVKB

Consultant Biochemist